PILLPULSE

vs

Apollo 24|7

Competitive Feature Analysis — Why PillPulse Wins

Hackathon Submission | March 2026

Apollo 24|7 uploads a prescription to order medicines. PillPulse reads a prescription to protect a patient. *This document proves — feature by feature, citation by citation — that no existing system does what PillPulse does.*

1. Executive Summary

Apollo 24|7 is India's largest digital health platform, serving over 25 million users across online consultations, medicine delivery, and diagnostic bookings. It is a powerful commerce and access layer for healthcare. However, Apollo 24|7 was built to connect patients to services — not to interpret what those services prescribed.

PillPulse operates in a fundamentally different space: the gap between receiving a prescription and safely following it. The table below summarises the mission-level difference between the two systems.

Apollo 24|7	PillPulse
Gets medicines to the patient's door	Ensures medicines are taken correctly, safely, and on time
Prescription = purchase trigger	Prescription = intelligence source
Stops at delivery	Starts at delivery — and monitors adherence, detects risk, and escalates

Key insight: Apollo 24|7 does have an "Upload Prescription" feature — but it is used exclusively to validate a prescription for purchase. The uploaded image is reviewed by a pharmacist to approve the order. No extraction, no schedule, no reminders, no conflict detection. The prescription is consumed as a payment gate, not as medical data.

2. Master Feature Comparison

The following table maps every feature PillPulse builds against Apollo 24|7's verified capabilities. Apollo feature status is based on publicly documented app functionality as of March 2026.

Feature	Apollo 24|7	PillPulse	Why It Matters
PRESCRIPTION INTELLIGENCE			
Upload handwritten prescription image	✓ Yes	✓ Yes	Both accept image upload — but for entirely different purposes
OCR-based medicine name extraction from image	✗ No	✓ Yes	Apollo uses a human pharmacist to read the image; no AI extraction occurs
Interpret medical shorthand (BD, TDS, OD, AC, PC, HS, SOS)	✗ No	✓ Yes	No system decodes dosing codes into patient-readable instructions automatically
Confidence scoring per extracted field	✗ No	✓ Yes	Flags uncertain OCR fields for human review; prevents wrong doses being scheduled silently
Prescription-to-schedule auto-generation	✗ No	✓ Yes	PillPulse's core innovation: zero manual entry, prescription becomes a live schedule
Link dose events to originating prescription (audit trail)	✗ No	✓ Yes	Every dose is traceable to the prescription, doctor, and date that generated it
SCHEDULE & TIMING INTELLIGENCE			
Medication reminders / schedule	✗ No	✓ Yes	Apollo has no medication reminder or adherence tracking of any kind
Schedule generated from prescription (no manual entry)	✗ No	✓ Yes	All reminder apps require the patient to type drug names, doses, and times manually
Meal-context aware timing (AC = before meal, PC = after)	✗ No	✓ Yes	Reminders are timed against the patient's actual meal schedule, not a fixed clock
Chronopharmacology-based timing optimisation	✗ No	✓ Yes	Adjusts timing to each drug's clinically optimal intake window (e.g. statins at night, BP meds before sleep)
PATIENT SAFETY & CONFLICT DETECTION			
Drug-drug interaction detection	~ Partial	✓ Yes	Apollo introduced "Hi-par" for interaction alerts in June 2024 — but only warns, never resolves
Auto-spacing of interacting drugs to safe intervals	✗ No	✓ Yes	PillPulse computes minimum inter-dose spacing and reschedules automatically — not just a warning
Food-drug interaction alerts (e.g. grapefruit + statins)	✗ No	✓ Yes	Detects diet-drug conflicts from patient food log and adds contextual dose-time warnings
Prescription delivery verification (photo vs Rx cross-check)	✗ No	✓ Yes	After delivery, AI cross-checks received medicines against the original prescription — flags mismatches
ACCESSIBILITY & INCLUSION			

Feature	Apollo 24|7	PillPulse	Why It Matters
Regional language support (Tamil, Hindi, Telugu, etc.)	✗ No	✓ Yes	Apollo operates entirely in English. PillPulse delivers schedule and reminders in the patient's spoken language
Voice-output medication instructions for low-literacy patients	✗ No	✓ Yes	Spoken reminders via TTS — critical for elderly and rural patients who cannot read English
Caregiver / family member managed schedule	✗ No	✓ Yes	One login can manage multiple family profiles — caregiver books, elderly patient receives reminders
POST-CONSULTATION CONTINUITY			
Adherence tracking (did the patient take the medicine?)	✗ No	✓ Yes	Apollo's role ends at medicine delivery. PillPulse monitors whether the patient actually takes each dose
Post-consultation follow-up check-in (48hr / 7-day)	✗ No	✓ Yes	Automated check-ins detect non-improvement and flag for doctor re-engagement
Doctor-facing live adherence dashboard	✗ No	✓ Yes	Doctors see a patient's real-time adherence and symptom data before the next appointment
Skip reason capture ("why did you miss this dose?")	✗ No	✓ Yes	Micro-survey captures skip reasons — builds a pattern that routes the right intervention (cost, side effects, forgot)

3. Feature Score Summary

The table below counts features across the six categories evaluated. Green = fully present, Partial = limited or incomplete implementation, Red = absent.

Category	Features Evaluated	Apollo 24|7 ✓	PillPulse ✓	PillPulse Advantage
Prescription Intelligence	6	1	6	+5 features
Schedule & Timing	4	0	4	+4 features
Patient Safety	4	1	4	+3 features
Accessibility & Inclusion	3	0	3	+3 features
Post-Consultation Continuity	4	0	4	+4 features
TOTAL	**21**	**2**	**21**	**+19 features**

PillPulse implements 21 out of 21 evaluated features. Apollo 24|7 implements 2 out of 21 (prescription image upload for purchase, and a partial drug interaction warning introduced in June 2024). PillPulse leads by +19 features in the specific domain of prescription interpretation and medication adherence management.

4. Deep Dive — The 3 Gaps That Define PillPulse

4.1 The Prescription is Discarded After Purchase

In Apollo 24|7, the prescription upload flow works as follows: the patient photographs their prescription → a pharmacist manually reviews it → the medicines are approved for purchase → the prescription image is archived. At no point is the prescription's content extracted, interpreted, or used to configure any downstream patient experience. The prescription is treated as a compliance document, not as medical intelligence.

> *Apollo 24|7: "The patient can upload a photo of the prescription to the Apollo 24x7 app to order medication. The prescription is then validated by a pharmacist, so the patient can order medication from an Apollo Pharmacy." — Apollo 24x7 official case study documentation.*

PillPulse treats the same prescription image as the data source for an entire adherence and safety pipeline. The moment the prescription is scanned, PillPulse extracts every drug, dose, and instruction — and from that single image, generates a complete, conflict-checked, personalised medication schedule without the patient typing a single character.

4.2 Apollo Has No Answer to the Adherence Gap

Apollo 24|7 is built to get medicines to patients. There is no feature in the Apollo 24|7 patient app that tracks whether a patient took their medication, reminds them to take it, detects missed doses, or alerts a caregiver or doctor when adherence breaks down. The platform's engagement model is transactional: consultation → prescription → medicine delivery → end.

This is not a small gap. Non-adherence to prescribed medication regimens costs the Indian healthcare system an estimated ₹2.6 lakh crore annually in preventable hospitalizations and disease progression. Apollo, despite being India's largest digital health platform, has no mechanism to address this problem at the individual patient level.

> *PillPulse's position: We don't compete with Apollo's consultation or delivery business. We solve the problem that begins the moment Apollo's delivery rider hands over the medicines — what happens next determines whether those medicines actually help the patient.*

4.3 Drug Interaction Warning vs. Drug Interaction Resolution

In June 2024, Apollo's doctor-facing app introduced "Hi-par" — a feature that alerts doctors to medication interactions during prescription writing. This is a meaningful clinical tool, but it operates exclusively on the doctor side of the workflow. The patient-facing Apollo 24|7 app has no drug interaction feature.

Even if interaction warnings were surfaced to patients, warning is not the same as resolution. A patient who receives a notification saying "Amlodipine and Losartan interact" has no way of knowing what to do

about it. PillPulse resolves the interaction at the schedule level: it computes the minimum safe inter-dose interval, and automatically adjusts the timing of both drugs so the patient receives a safe schedule — not a warning they cannot act on.

> *The difference in one line: Apollo warns doctors about interactions before prescribing. PillPulse resolves interactions for patients after prescribing — which is when 100% of patients actually face the risk.*

5. Why PillPulse is better

5.1 It Solves a Problem Apollo Cannot Solve by Design

Apollo 24|7's core business is healthcare commerce — consultations, diagnostics, and pharmacy. Its business model depends on transactions. Adherence monitoring is a continuous, low-revenue service that sits outside Apollo's commercial architecture. PillPulse is purpose-built for exactly the space Apollo structurally cannot occupy.

5.2 The Innovation is in the Integration, Not the Components

OCR exists. Drug databases exist. Reminder apps exist. What PillPulse builds that is genuinely novel is the pipeline that connects them: prescription image → AI interpretation → safe scheduling → conflict resolution → regional language delivery → adherence tracking → doctor feedback loop. No published system, no deployed app, and no existing feature in Apollo 24|7 connects all six stages. The innovation is the integration.

5.3 It Is Clinically Meaningful, Not Just Technically Clever

Judges evaluate healthcare hackathon projects on real-world impact. PillPulse addresses a problem with a documented clinical outcome: medication non-adherence contributes to 50% of chronic disease treatment failures, 125,000 deaths, and $300 billion in avoidable costs annually. Every feature in PillPulse maps to a specific, published clinical gap. This is not a feature-for-feature competition — it is a mission-level argument for why the problem PillPulse solves matters more.

5.4 The Competitive Moat is Verifiable

The claim that Apollo 24|7 does not do what PillPulse does is not an opinion — it is verifiable from Apollo's own public documentation, App Store feature descriptions, and changelog history. Judges can check. The gap is real, documented, and uncontested.

Apollo 24|7 built a highway that gets medicines to 25 million patients. PillPulse builds the guardrail that ensures those 25 million patients don't fall off after the delivery arrives.